The Royal New Zealand College of General Practitioners
Te Whare Tohu Rata o Aotearoa

Clinical record review self-audit checklist

Introduction

General practices deliver a service that must be managed effectively to ensure that it meets the needs of patients. The patient's clinical record is integral to maintaining good patient care and continuity of care.

Patient records should describe and support the health care that has been provided. They should be understandable to a newcomer to the practice. The structure of the records should allow information to be obtained easily. It is important that adequate information is recorded for each consultation and that the person making the entry is identified – this includes telephone consultations.

Conducting a record review helps to establish and improve the quality of clinical records and supports the safe care of patients.

This clinical record review self-audit checklist can be used by practices that are conducting self-assessments in preparation for practice accreditation processes. The checklist enables doctors, practice nurses and practices to ensure that they are meeting Indicator 21 of *Aiming for Excellence: The RNZCGP standard for New Zealand general practice*: 'Patient records meet requirements to describe and support the management of health care provided'. The criteria in this checklist also link to a number of other criteria in the Standard.

This clinical record review self-audit checklist is also a component of the General Practice Education Programme (GPEP) and can be used as an audit activity for the continuing professional development programme (CPD/MOPS).

The tool has two parts:

- **MODULE 1** includes requirements for the practice patient record system and the demographic details that it records. This module should be undertaken for **practice accreditation purposes**.

  If this module has been done in the practice within the past three years, it does not need to be completed by doctors undertaking Module 2 for the training programme or for professional development purposes. The module contains both foundational and advanced criteria.

- **MODULE 2** contains criteria that should be assessed for **practice accreditation, training programme and professional development purposes**. The module contains both foundational and advanced criteria.

The bullets alongside criteria in the tool indicate the following:

- F These are basic criteria that must be assessed in the **Foundation Standard** process. These criteria indicate that the clinical record meets minimum legal, regulatory and professional standards and is sufficient for safe practice. These criteria are found in both modules 1 and 2.

- A These are advanced criteria that must be assessed as part of the **CORNERSTONE®** assessment process. These criteria indicate that the clinical record is complete. These criteria are found in both modules 1 and 2.

Both basic and advanced criteria are completed for the **training and continuing professional development programmes**.

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Instructions

Modules 1 and 2 of the checklist require a random audit of **10** patient records.* Applications that generate lists of random numbers are available online. However, the easiest way to generate a random sample is to select consecutive patient appointments, beginning at a random time on a randomly selected day.

Patient records should be electronic. All records selected should have an entry in the past 12 months. The review should not focus on a single consultation but rather on a series of the most recent consultations for a particular record.

Module 1 can be completed by practice administration staff. Module 2 must be completed by the clinician whose notes are being audited.

Modules 1 and 2

- Randomly select 10 patient clinical records.
- Complete the Module 1 or Module 2 template attached by marking the boxes in the columns numbered 1 to 10 for each of the records reviewed as follows:
  - **Y** present and adequate
  - **IN** present but inadequate
  - **N** not present
  - **N/A** not applicable/necessary in this case
- Evaluate each of the criteria by selecting 'met', 'part met', 'not met' or 'n/a' (not applicable) for each of the rows.
- Complete the Report and Plan template, identifying areas for development and a plan for improvement.

GPEP registrars

It is your responsibility to check whether the practice has completed Module 1 of the checklist in the past three years, and if not, to compete it as part of your audit requirements. If your practice has completed Module 1, please provide a copy with your completed Module 2.

You are required to submit the completed clinical record audit checklist to the College. Complete and return both the Recording Sheet and the Report and Plan sheet to your GPEP Programme Advisor. **You are encouraged to discuss the results of this audit activity with your medical educator, collegial relationship provider, or a peer**.

* For Foundation Standard and CORNERSTONE® purposes, each clinician in the practice must complete a self-audit of 10 records.

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Clinician's name: Ryotaro Eguchi

MCNZ/NCNZ number: 69327

Date: 16/04/26

MODULE 1

Patient record system†

	Record:	1	2	3	4	5	6	7	8	9	10	Met	Part met	Not met	N/a
1.	**Patient records are electronic, secure and traceable:**														
	All clinical information is:														
●	recorded electronically	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	password protected	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	reliably backed up	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Clinical notes:														
●	are dated	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	reliably identify the author	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
2.	**Basic demographic information is sufficient to allow for patient identification and to meet national enrolment requirements:**														
	Information stored for each patient includes:														
●	NHI number	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	name	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	gender	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	address	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	date of birth	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	ethnicity	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	registration status	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Information held for enrolled patients includes:														
●	contact phone number	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	contact in case of emergency (ICE)	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	next of kin – where applicable	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	significant relationships	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	hapū/iwi for Māori patients	NA	Y	NA	NA	NA	Y	NA	Y	Y	Y	✓			
●	primary language if not English	NA	NA	NA	Y	NA	NA	Y	Y	Y	Y	✓			
	Need for an interpreter:														
●	Any need for an interpreter is flagged for patients with English as a second language	NA	NA	NA	Y	NA	NA	Y	NA	NA	NA	✓			

† This module is completed at practice level as part of the Foundation Standard and CORNERSTONE® accreditation process. Provided it has been done at practice level in the past three years, it does not need to be completed by doctors undertaking Module 2 for other purposes. If not doing this section themselves, GPEP2/3 registrars must attach a copy of the completed Module 1 done by their practice.

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Clinician's name:

MCNZ/NCNZ number:

Date:

MODULE 2

Clinical record review

	Record:	1	2	3	4	5	6	7	8	9	10	Met	Part met	Not met	N/a
1.	**The record is appropriate, contemporaneous and sources are identified:**														
●	Notes are completed as soon as possible after contact, and any delay is identifiable.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Information is recorded objectively and does not contain inappropriate, judgmental comment.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	When information is provided other than by the patient, the source is identified.	Y	Y	Y	Y	Y	N	Y	Y	Y	Y		✓		
2.	**Clinical notes can be understood by someone not working regularly at the practice:**														
●	The notes are logical, intelligible and sequential.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	The use of keywords or templates does not compromise the validity of the notes.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
3.	**Important background issues, warnings and alerts are displayed for all records:**														
●	Past medical history is available.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Significant social history is included.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	The PMS is used to effectively display important warnings, and alerts.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Allergies or the absence of known allergies is recorded for each patient.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
4.	**Specific patient needs and instructions are recorded and are available in easily accessible form at the clinically relevant point:**														
●	Patient needs recorded include any directives by patient, disabilities, drug dependencies, end of life and special needs (eg communication, mental health issues).	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
5.	**The recorded history is relevant and sufficient for both safe management and evidential purposes:**														
●	The reason(s) for the encounter recorded or apparent from the notes.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	The record includes date, place of consultation (if different from usual) and mode of contact if not face to face.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
6.	**The record includes all findings essential to diagnosis and management:**														
●	Sufficient positive and negative history and examination findings are present to justify management decisions.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Objective measurements (BP, pulse, temp., respiratory rate, PaO2 etc) are recorded, where relevant.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			

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	Record:	1	2	3	4	5	6	7	8	9	10	Met	Part met	Not met	N/a
7.	**The working diagnosis/differential or problem being managed is apparent and consistent with supporting information:**														
●	The diagnosis (and any differential) and level of certainty is clear from the notes.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
8.	**The patient management plan is clear and identifies and addresses uncertainty and conjecture:**														
●	The plan for care can be identified from the record.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Important assumptions and remaining uncertainties in diagnosis and management are noted.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
9.	**The record identifies information given to the patient, including risks and benefits of treatments and, where relevant, consent:**														
●	Notification of test results and clinical findings is recorded.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	The record supports adequate consenting processes.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
10.	**All important clinical decisions and interventions are recorded:**														
●	Treatment plans, including interventions, contingency plans, safety netting and follow-up arrangements are recorded as necessary.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Clinical management decisions made outside consultations (eg telephone calls) and off-site contacts (home visit, aged care facilities etc) are recorded.	NA	NA	Y	Y	Y	NA	Y	Y	Y	Y	✓			
11.	**The record identifies all medication treatment provided, including the type, dosage and total amount of any medications prescribed:**														
●	There is a record of all prescriptions issued, including drug name, administration instructions and quantities ordered.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
●	Medications initiated or changed outside the practice are reconciled with the PMS.	Y	NA	Y	NA	Y	NA	Y	Y	Y	Y	✓			
●	Current and long-term medications are differentiated and the status is clear.	NA	NA	Y	NA	Y	N	Y	Y	Y	Y		✓		
●	Where long-term medications are changed, reasons for alteration or discontinuation are clear.	NA	NA	Y	NA	Y	NA	N	Y	Y	Y		✓		
12.	**The record identifies all investigations requested and tracks high-risk tests:**														
●	All requests for tests and investigations are recorded.	Y	Y	Y	NA	Y	Y	Y	Y	Y	Y	✓			
●	High-risk tests (eg histology, cervical smears) are tracked for completion.	Y	NA	Y	NA	Y	NA	Y	Y	Y	Y	✓			
13.	**The record supports effective and timely referral for treatment or transfer of care:**														
●	The record shows that referrals are completed within a reasonable time frame.	NA	NA	Y	NA	Y	NA	Y	Y	Y	Y	✓			
●	Copies of referral letters to and from the practice, certifications, referrals and responses, discharge summaries and test results are included in the patient PMS record or accessibly filed.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			

	Record:	1	2	3	4	5	6	7	8	9	10	Met	Part met	Not met	N/a
	Referrals include urgency, reason/ expectation of referral, relevant findings, classifications, warnings and current treatment.	NA	NA	Y	NA	Y	Y	Y	Y	Y	Y	✓			
	The transfer of responsibility for care can be verified from the records.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
14.	**Follow-up of test results is clearly documented and actions recorded:**														
	Follow-up actions on test results and referrals are recorded.	Y	Y	Y	NA	Y	Y	Y	Y	Y	Y	✓			
15.	**Screening history and results (or declined screening) are recorded:**														
	Screening history and results (including declines) are evident for routine screening areas (eg cervical smears, mammograms, cardiovascular risk assessment, diabetes screening).	Y	Y	Y	NA	Y	Y	Y	Y	Y	Y	✓			
	Screening recall status can be easily tracked.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	There is evidence of patient risk assessment and opportunistic screening for high-risk conditions.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
16.	**Immunisation history and status is recorded:**														
	There is evidence that recommended immunisations are provided in accordance with the national schedule.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Records show advice given and immunisation status for non-scheduled immunisations.	N	N	Y	N	Y	Y	Y	Y	Y	Y		✓		
17.	**There is a systematic record of individual risk factors:**														
	Diseases are classified for chronic conditions, including all conditions for which the patient is on long-term treatment.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Family history for major risk factors, such as diabetes, early CVD, bowel and breast cancer etc.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Current employment (where relevant) and any history of at-risk occupations.	NA	NA	Y	NA	N	Y	N	Y	Y	Y			✓	
	Blood pressure monitoring as clinically indicated.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Baseline weight/BMI and monitoring as clinically indicated.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Smoking status and history and cessation support offered, where relevant.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Alcohol and drug usage.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y	✓			
	Regular review of chronic conditions as per current best practice (eg INR, diabetes, CVR).	NA	NA	Y	NA	Y	Y	Y	Y	Y	Y	✓			

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Report and Plan template

After having completed modules 1 and/or 2 of the tool, summarise your findings and plan any necessary improvements on the template below. The template lists each of the indicators in modules 1 and 2. In column 1 under each indicator, record the proportion of records that did not meet the criteria. In column 2, give the areas that need improvement, and in column 3, briefly describe your plan for improvement.

Audit finding:	Area(s) of omission:	Plan for improvement:
Module 1		
1. Patient records are electronic, secure and traceable:		
0/10	Nil	Continue current practice.
2. Basic demographic information is sufficient to allow for patient identification and to meet national enrolment requirements:		
0/10	Nil	Continue current practice.
Module 2		
1. The record is appropriate, contemporaneous and sources are identified:		
1/10	In one record involving a young teenager, history was provided by the patient's mother. The source of information was not identified in the clinical note.	When history is obtained from a third party (parent, caregiver, or other), explicitly document the source at the start of the encounter note (e.g. "History from mother" or "History provided by parent").
2. Clinical notes can be understood by someone not working regularly at the practice:		
0/10	Nil	Continue current practice.
3. Important background issues, warnings and alerts are displayed for all records:		
0/10	Nil	Continue current practice.

Audit finding:	Area(s) of omission:	Plan for improvement:
4. Specific patient needs and instructions are recorded and are available in easily accessible form at the clinically relevant point:		
0/10	Nil	Continue current practice.
5. The recorded history is relevant and sufficient for both safe management and evidential purposes:		
0/10	Nil	Continue current practice.
6. The record includes all findings essential to diagnosis and management:		
0/10	Nil	Continue current practice.
7. The working diagnosis/differential or problem being managed is apparent and consistent with supporting information:		
0/10	Nil	Continue current practice.
8. The patient management plan is clear and identifies and addresses uncertainty and conjecture:		
0/10	Nil	Continue current practice.
9. The record identifies information given to the patient, including risks and benefits of treatments and, where relevant, consent:		
0/10	Nil	Continue current practice.
10. All important clinical decisions and interventions are recorded:		
0/10	Nil	Continue current practice.

Audit finding:	Area(s) of omission:	Plan for improvement:
11. The record identifies all medication treatment provided, including the type, dosage and total amount of any medications prescribed:		
2/10 (A criteria)	In one record, medications were documented but not differentiated as long-term versus current/acute. In a second record, a medication was changed but the reason for alteration was not documented.	Ensure the PMS medication list distinguishes long-term/repeat medications from acute/short-course prescriptions. When stopping or changing a long-term medication, document the clinical reason in the consultation note.
12. The record identifies all investigations requested and tracks high-risk tests:		
0/10	Nil	Continue current practice.
13. The record supports effective and timely referral for treatment or transfer of care:		
0/10	Nil	Continue current practice.
14. Follow-up of test results is clearly documented and actions recorded:		
0/10	Nil	Continue current practice.
15. Screening history and results (or declined screening) are recorded:		
0/10	Nil	Continue current practice.
16. Immunisation history and status is recorded:		
3/10 (A criteria)	Non-scheduled immunisation status was not discussed or documented in 3 records. This included shingles vaccine for patients aged 50+ and meningococcal vaccine for teenage patients, where clinically indicated.	For patients aged 50 and over, proactively discuss shingles vaccination and document advice given and the patient's decision. For teenage patients, discuss and document meningococcal vaccination status. Record in the clinical note at the time of the consultation.
17. There is a systematic record of individual risk factors:		
2/10 (A criteria)	Occupational history was not documented in 2 records where it may have been relevant. Employment and at-risk occupation history is not routinely asked unless the consultation is ACC-related.	Where clinically relevant (e.g. musculoskeletal, respiratory, or occupational health presentations), include employment and occupational history in the clinical record. Consider adding occupational history to the new patient registration template.